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Attorneys for Plaintiffs

The Honorable Thomas O Rice

IN THE UNITED STATES DISTRICT COURT
FOR THE EASTERN DISTRICT OF WASHINGTON
AT SPOKANE

JANE ELIZABETH ROBERTS, *et al,*
Plaintiffs,

v.

SHRINERS HOSPITALS FOR CHILDREN, SHRINERS HOSPITALS FOR CHILDREN-SPOKANE, BEVERLY BOKOVITZ, FRANCES FARLEY, JERRY GANTT, JOHN McCABE, PHILLIP GRADY, PETER BREWER, AND GOV. JAY INSLEE
Defendants.

No. 2:23-CV-295-TOR

PLAINTIFFS' OPPOSITION TO GOV. JAY INSLEE'S MOTION TO DISMISS

March 6, 2024

ORAL ARGUMENT REQUESTED

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TABLE OF CONTENTS

I. Introduction 5
II. Legal Standard 6
III. Statement of Fact 7

A. Defendant's Response to the COVID-19 Pandemic 7
B. The Delta Surge 7
C. Unsuccessful Legal Challenges 8
D. *Curtis v. Inslee* and *Johnson v. Brown* 8
E. Plaintiffs' § 1983 Claims 9
1. Rights Creating Language 9
2. Suing Inslee in his Official Capacity 11
3. § 1983 Claims 12
a. Investigational Drug Use Claim 12
b. The EUA Act Does Apply 17
c. Enforceable Rights 19
i. 45 CFR 46 19
ii. The Belmont Report 20
iii. The FWA 20
d. Due Process 20
i. Substantive Due Process Claims 20
ii. Procedural Due Process Claims 22
e. Equal Protection 23
f. Spending Clause 24
g. Unconstitutional Conditions Doctrine 24
h. The PREP Act 25
F. State Law Claims 26
1. Breach of Contract 26
2. Outrage 27
G. Implied Right of Action 28
H. Qualified Immunity 28

IV. Oral Argument Requested 33

V. Conclusion 33

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TABLE OF AUTHORITIES

Cases

Anderson v. Creighton, 483 U.S. 635 (1987) 33
Becker v. Community Health Systems, 359 P.3d 746 (2015) 27
Daniels-Hall v. Nat'l Educ. Ass'n, 629 F.3d 992 (9th Cir. 2010) 16
Groten v. California, 251 F.3d 844 (9th Cir. 2001) 31
Holloman v. Harland, 370 F.3d 1252 (11th Cir. 2004) 31
Johnson v. Duffy, 588 F.2d 740 (9th Cir. 1978) 29
Logan v. Zimmerman Brush Co., 455 U.S. 422 (1982) 23
Mack v. S. Bay Beer Distribs., Inc., 798 F.2d 1279 (9th Cir. 1986) 16
Malley v. Briggs, 475 U.S. 335 (1986) 29
Metzler Investment v. Corinthian Colleges, Inc., 540 F.3d 1049 (9th Cir. 2008) .. 16
Milliken v. Meyer, 311 U. S. 457 (1940) 23
Modglin v. DJO Glob. Inc., 114 F. Supp. 3d 993, 1008 (C.D. Cal. 2015) 16
Rickman v. Premera Blue Cross, 358 P.3d 1153 (2015) 27
Rose v. Anderson Hay & Grain Co., 358 P.3d 1159 (2015) 27
Roska v. Sneddon, 437 F.3d 964 (10th Cir. 2006) 31
S. Bay United Pentecostal Church v. Newsom, 985 F.3d 1128 (9th Cir. 2021) 16
Silberstein v. City of Dayton, 440 F.3d 306 (6th Cir. 2006) 30
Sims v. Adams, 537 F.2d 829 (5th Cir. 1976) 29
Sylvia Landfield Tr. v. City of Los Angeles, 729 F.3d 1189 (9th Cir. 2013) 21
United States v. DJO Glob., Inc., 678 F. App'x 594 (9th Cir. 2017) 16
United States v. Lanier, 520 U.S. 259 (1997) 33

Statutes

10 U.S.C. § 980 25
21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III) 19, 23, 26
21 U.S.C. §360bbb-3(l) 20
42 U.S.C. § 1983 26
45 C.F.R. § 46 ..... 20, 21

Other Authorities

Justice Department Announces Largest Health Care Fraud Settlement in Its History. Justice.gov. Published September 2, 2009 33

Rules

Fed. R. Evid. 201(b) 16

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I. INTRODUCTION

The federal government purchased all of the COVID-19 EUA/PREP Act drugs. It sought the help of Washington State distribute these federally owned drugs under the terms and conditions of the CDC COVID-19 Vaccination Program Provider Agreement ("Provider Agreement"). Governor Inslee volunteered the State to assume those duties on behalf of the federal government. The State was legally responsible for the CDC COVID-19 Vaccination Program ("CDC Program") and recruited public and private parties to help distribute the drugs. Therefore, the State and recruited parties owed constitutional obligations to persons offered the federal COVID-19 property.

Governor Inslee's executive department required every party to sign and abide by the Provider Agreement. The Provider Agreement required each party to comply with 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III) (option to accept or refuse) and with "any EUA." Moreover, any administration outside the scope of the CDC Agreement or any EUA would constitute fraud against the federal government.

Governor Inslee, having required EUA "vaccination providers" and Provider Agreement "Organizations" to ensure all individuals under his authority were informed of their lawful authority to accept or refuse by signing the Provider

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Agreement, issued his Proclamation requiring those same parties to commit fraud against the federal government and usurp from the U.S. Congress its authority to prohibit the involuntary use of the federally owned investigational new drugs. To escape liability for his actions, the Defendant comes before this court and claims his actions were rational, lawful, and in line with court precedent across the nation because he asserts that the investigational new drug could be promoted as COMIRNATY® despite the FDA, CDC, HHS, and Pfizer informing him otherwise. Moreover, Defendant issued his Proclamation weeks before the FDA approved any drug with a legal indication to prevent the COVID-19 virus.

Jay Inslee, the individual, acted outside the scope of his authority, willfully destroying the lives of healthcare professionals whose only crime was exercising their option to refuse under the watchful eye of the Fourteenth Amendment.

II. LEGAL STANDARD

A Complaint does not require detailed factual allegations; it simply must plead "enough facts to state a claim to relief that is plausible on its face." *Bell Atlantic Corp. v. Twombly*, 550 U.S. 544, 570 (2007). When considering a Rule 12 Motion to Dismiss, the Court is required to accept Plaintiffs' factual allegations as true. *Ashcroft v. Iqbal,* 556 U.S. 662 (2009).

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The defendant cites many cases holding that a person had the authority to mandate a true vaccine during the COVID-19 pandemic. Those cases are not applicable because Plaintiffs in the instant action do not challenge that authority. Moreover, the cited cases did not consider the facts presented herein (an EUA/PREP Act drug mandate) and thus provided no judicial support for Defendant's Motion to Dismiss the Plaintiffs' valid claims.

III. STATEMENT OF FACTS

A. Defendant's Response to the COVID-19 Pandemic

Gov. Inslee recites his response to the pandemic. Still, it is immaterial to the instant action because the U.S. Congress created the EUA statute and the PREP Act to be executed only during a declared emergency. Still, Congress expressly prohibited involuntary participation in products or activities authorized under the statutes. Defendant acted under *ultra vires* authority when amending the conditions established by the FDA and HHS requiring that which Congress prohibited—involuntary participation.

B. The Delta Surge

Defendant's reliance on the "Safety Review Workshop" provides no cover for Defendant's unlawful Proclamation because the workgroup had no authority to

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override the U.S. Congress, FDA, HHS, CDC, or Pfizer. Each manufacturer bears responsibility for ensuring its product is administered according to its EUA. That fact is one reason that Congress completely preempts states from the field of its emergency expanded access protocols under the EUA Statute. *Biden v. Missouri*, 595 U.S. 87, 89 (2022) (citing 86 Fed. Reg. 61555 (2021)) only held that the vaccine mandate was lawful, not that an EUA/PREP Act drug mandate was lawful. The federal government did not issue its Provider Agreement only to require each person signing that agreement to commit fraud against the federal government by violating its terms but being compensated anyway. Such an implication is nonsensical.

C. Unsuccessful Legal Challenges

The cited cases are immaterial to the instant action because they do not speak to the statutory rights of individuals to refuse an EUA/PREP Act drug and how the Fourteenth Amendment protects that right under a § 1983 cause of action.

D. *Curtis v. Inslee* and *Johnson v. Brown*

The *Curtis* court did not rule on whether the drugs were federally funded, investigational, under EUA, PREP Act, or the CDC Program and how those facts impacted Plaintiffs' claims. The failure of the *Curtis* court to accept the Plaintiffs' allegations and federal government documentation as true caused the court to rule

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in error. It provided this court with no judicial reasoning to dismiss the Plaintiffs' claims against Governor Inslee. In addition to Judge Bryan's ruling being nonbinding on this court, it is currently the subject of a Motion to Alter or Amend, and, failing that, will be appealed to the 9th Circuit Court of Appeals, as undersigned counsel represents the plaintiffs in *Curtis*.

Gov. Inslee also incorrectly relies upon the case of *Johnson v. Brown,* 567 F.Supp.3d 1230 (D. Or. 2021) in footnote 12, and pages 15, 18, 24, 25, 26, and 30,. First, Judge Simon's ruling from the Oregon District Court is not binding on this Court. Second, the case brought before Judge Simon is not based on the same cause of action as the case at bar, so that ruling has no bearing on the Constitutional and federal statutory issues raised herein. Third, the same facts and law were not presented to Judge Simon.

E. Plaintiffs' § 1983 Claims

1. Rights Creating Language

Plaintiffs aver that rights-creating language, as discussed in *Health & Hosp. Corp. of Marion Cnty. v. Talevski*, 599 U.S. 166, 183 (2023), exists in both the EUA statute (the option to accept to refuse) and the PREP Act (the right to be informed of the voluntary nature of the program and the right to refuse under the EUA

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statute), deprivation of which forms the basis of an enforceable § 1983 action. Moreover, the very recent 7-2 Supreme Court decision in *Talevski* involved the Spending Clause and contracts issued by the federal government under that Clause, having rights creating language for individuals in the spending statute, as in the case at bar.

The *Talevski* Court held,

> *Gonzaga* sets forth our established method for ascertaining unambiguous conferral. Courts must employ traditional tools of statutory construction to assess whether Congress has "unambiguously conferred" "individual rights upon a class of beneficiaries" to which the plaintiff belongs. [citations omitted] Notably, it must be determined that "Congress intended to create a federal right" *for* the identified class, not merely that the plaintiffs fall "within the general zone of interest that the statute is intended to protect" *Gonzaga*, 536 U.S., at 283. This paradigm respects Congress's primacy in this arena and thus vindicates the separation of powers. *Id*., at 286. Id., at 284, 287. Conversely, we have rejected § 1983 enforceability where the statutory provision "contain[ed] no rights-creating language"; had "an aggregate, not individual, focus"; and serve[d] primarily to direct the [Federal Government's] distribution of public funds." *Id*., at 290
>
> We have held that the *Gonzaga* test is satisfied where the provision in question is "phrased in terms of the persons benefited" and contains "rights-creating," individual-centric language with an "unmistakable focus on the benefited class."

The Court held that the "right to be free from . . . any physical or chemical restraints" and the right to advanced notice of discharge provisions of the FNHRA

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statute "meet[s] this test," stating that "This framing is indicative of an individual "rights-creating" focus. *Gonzaga*, 536 U. S., at 284."

While the federal statutes involved in the instant action and *Talevski* differ, the legal analysis is identical. Plaintiffs at bar claim that Congress created a right for them under 21 U.S.C. 360bbb-3(e)(1)(A)(ii)(III), in pertinent part:

> The Secretary…shall establish…conditions…to ensure that individuals to whom the product is administered are informed…of the option to accept or refuse administration of the product…

This language "unambiguously confers" the right to be informed of the option to accept or refuse administration of the product. Also, it speaks in terms of "individual rights upon a class of beneficiaries" to which the plaintiff belongs. The provision uses the word "individuals" when describing to whom the right is conferred. The beneficiaries are those contemplating the "administration of the product." By describing the right the way it did, Congress intended to create a federal right for the identified class.

2. Suing Inslee in His Official Capacity

Plaintiffs agree that Governor Inslee cannot be sued under § 1983 in his official capacity. He can, though, be sued in his personal capacity.

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3. § 1983 Claims

For reasons discussed above and in *Talevski*, the Defendant is incorrect when asserting that the Plaintiffs cannot sue under under § 1983 for the deprivation of rights created under the EUA statute.

a. Investigational Drug Use Claim

Referring to the "Pfizer vaccine" only sows confusion because drugs are regulated according to their labeling (21 U.S.C. §§ 331,355). "Pfizer BioNTech vaccine" (lowercase v) refers to Pfizer's COVID-19 vaccine technology. "Pfizer-BioNTech COVID-19 Vaccine" (upper case V) refers to Pfizer's formal name for its investigational new drug under application 19736. "COMIRNATY®" is Pfizer's only FDA-licensed COVID-19 drug. The Defendant disputes the FDA, not the Plaintiffs, when claiming the FDA did not classify the federally owned COVID-19 drugs as investigational and, therefore, fails. Black letter law prevents Pfizer-BioNTech COVID-19 Vaccine being granted a marketing license simply because another drug, sharing formulation, received its FDA-license. Defendant's attempt to apply COMIRNATY®'s label to the Pfizer-BioNTech COVID-19 Vaccine drug is the quintessential definition of misbranding (21 U.S.C. § 331).

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The FDA informed Governor Inslee on August 23, 2021, in its EUA letter, that the "Pfizer-BioNTech COVID-19 Vaccine" drug (Doc 1-2) is "legally distinct" from COMIRNATY® stating, "COMIRNATY (COVID-19 Vaccine, mRNA) is the same formulation as the Pfizer- BioNTech COVID-19 Vaccine" (emphasis added) (Doc 1-2, p.2, last paragraph) and "the products are legally distinct" (Doc 1-2, FN 8). In that same letter, the FDA issued its "Criteria for Issuance of Authorization" (Doc 1-2, p.5) because, among other things,

> "There is no adequate, approved, and available alternative to the emergency use of Pfizer-BioNTech COVID-19 Vaccine. Although COMIRNATY (COVID-19 Vaccine, mRNA) is approved to prevent COVID-19 in individuals 16 years of age and older, there is not sufficient approved vaccine available for distribution to this population in its entirety at the time of reissuance of this EUA. Additionally, there are no products that are approved to prevent COVID-19 in individuals age 12 through 15, or that are approved to provide an additional dose to the immunocompromised population described in this EUA."

On p. 10 of that same letter (Doc 1-2, Letters P, R, X), the FDA required the following:

> P. "Emergency response stakeholders[1] [i.e., the State] will

[1] "For purposes of this letter, "emergency response stakeholder" refers to a public health agency and its delegates that have legal responsibility and authority for responding to an incident, based on political or geographical boundary lines (e.g., city, county, tribal, territorial, State, or Federal), or functional (e.g., law enforcement or public health range) or sphere of authority to administer, deliver, or distribute vaccine in an emergency situation." (Doc 1-2, FN 12)

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> ensure that vaccination providers[2] [i.e., Shriners] within their jurisdictions are aware of this letter of authorization, and the terms herein and any subsequent amendment that might be made to the letter of authorization, **instruct** them about the means through which they are to obtain and administer the vaccine under the EUA, and ensure that the authorized labeling is made available to vaccination providers through appropriate means." (Emphasis added)
>
> R. "Vaccination provider will administer the vaccine in accordance with the authorization."
>
> X. "All descriptive printed matter, advertising, and promotional material relating to the use of the Pfizer-BioNTech COVID-19 Vaccine **clearly and conspicuously shall state that: This product has not been approved or licensed by FDA**, but has been authorized for emergency use by FDA…" (emphasis added)

Defendant's statement that "Plaintiffs falsely assert that '[n]o FDA-licensed COVID-19 vaccines have been introduced into commerce for general commercial marketing'" is patently false. An investigational new drug (IND) is assigned an identification number (e.g., IND 19736). By contrast, an FDA-licensed drug made available for general commercial marketing is assigned a National Drug Code

[2] "For purposes of this letter, "vaccination provider" refers to the facility, organization, or healthcare provider licensed or otherwise authorized by the emergency response stakeholder (e.g., non-physician healthcare professionals, such as nurses and pharmacists pursuant to state law under a standing order issued by the state health officer) to administer or provide vaccination services in accordance with the applicable emergency response stakeholder's official COVID-19 vaccination and emergency response plan(s) and who is enrolled in the CDC COVID-19 Vaccination Program." (Doc 1-2, FN 13)

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(NDC) that must be printed on the drug vials, containers, and promotional materials. (21 C.F.R. § 201) Pfizer received a license for its vaccine technology, and the FDA assigned it the marketing name "COMIRNATY" under NDC 0069-1000-XX. The CDC publishes a list of COVID-19 Vaccine-Related Codes, and manufacturers submit updates on the status of their FDA-licensed NDCs.[3] One type of update is when the NDC becomes "available" for general commercial marketing. According to Exhibit A, p. 4, dated September 28, 2022, a CDC report on COVID-19 drug codes, COMIRNATY® is listed in the section for NDCs "that will not be manufactured or made available in the near term even if authorized."[4] Pfizer confirmed CDC's report and is quoted on p. 4 of Exhibit A (Second Row, Last Column) stating,

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[3] https://www2a.cdc.gov/vaccines/iis/iisstandards/vaccines.asp?rpt=ndc

[4] While external to the Complaint, this document and others referenced in this Motion are properly subject to consideration on a motion to dismiss. See *Metzler Investment GMBH v. Corinthian Colleges, Inc.*, 540 F.3d 1049, 1061 (9th Cir. 2008) (court's review on motion to dismiss is limited to "the complaint, materials incorporated into the complaint by reference, and matters of which the court may take judicial notice"). Judicial notice is proper where the evidence "is not subject to reasonable dispute[.]" Fed. R. Evid. 201(b). Under this doctrine, a court on a motion to dismiss may consider, among other evidence, "records and reports of administrative bodies," *Mack v. S. Bay Beer Distribs., Inc.*, 798 F.2d 1279, 1282 (9th Cir. 1986), the content of state and federal regulations, *S. Bay United Pentecostal Church v. Newsom*, 985 F.3d 1128, 1132 (9th Cir. 2021), and public records and government documents available from reliable sources on the Internet, *U.S. ex rel. Modglin v. DJO Glob. Inc.*, 114 F. Supp. 3d 993, 1008 (C.D. Cal. 2015), aff'd sub nom. *United States v. DJO Glob., Inc.*, 678 F. App'x 594 (9th Cir. 2017); *Daniels-Hall v. Nat'l Educ. Ass'n*, 629 F.3d 992, 998-99 (9th Cir. 2010) (taking judicial notice of information on the websites of two school districts).

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> "Pfizer received initial FDA BLA license on 8/23/2021 for its COVID-19 vaccine for use in individuals 16 and older (COMIRNATY). At that time, the FDA published a BLA package insert that included the approved new COVID-19 vaccine tradename COMIRNATY and listed two new NDCs (0069-1000-03, 0069-1000-02) and images of labels with the new tradename. **These NDCs will not be manufactured**. Only NDCs for the subsequently BLA-approved tris-sucrose formulation will be produced." (Emphasis added)

Pfizer, the manufacturer of COMIRNATY®, informed the CDC that it never manufactured the original licensed version and would not manufacture it. That is why the FDA continued issuing EUAs for the Pfizer-BioNTech COVID-19 Vaccine drug – because Pfizer was not manufacturing, and did not manufacture, COMIRNATY® at any time before Gov. Inslee's deadline.

Defendant's statement that "This indisputably available 'Pfizer BioNTech' vaccine is the same vaccine that received full FDA approval on August 23, 2021" only serves to confuse the issue. "Pfizer BioNTech vaccine" (lowercase v) is Pfizer's vaccine technology and its licensed technology is marketed under the name of COMIRNATY®, not "Pfizer-BioNTech COVID-19 Vaccine." COMIRNATY® was approved but never distributed into the marketplace for general commercial marketing; thus, laws regulating FDA-approved drugs were never introduced into commerce.

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b. The EUA Act Does Apply

Defendant's assertion that the EUA statute does not apply fails. First, the governor volunteered the state to comply with the CDC Program, which requires all parties to comply with "any EUA." Second, Governor Inslee adopted the Provider Agreement as State Policy when establishing a requirement for any person desiring to join it in administering the federal property to sign it and comply with its provisions. Third, the Provider Agreement states, "Organization must comply with all applicable requirements as set forth by the U.S. Food and Drug Administration, including but not limited to requirements in any EUA that covers COVID-19 Vaccine." Last, Governor Inslee is an "emergency response stakeholder" under each EUA.

The August 23, 2021, EUA letter was explicit, as all others before and after were. The "Emergency response stakeholders [i.e., Governor Inslee] will ensure that vaccination providers [i.e., Shriners] within their jurisdictions are aware of this letter of authorization, and the terms herein and any subsequent amendment that might be made to the letter of authorization, **instruct** them about the means through which they are to obtain and administer the vaccine under the EUA, and ensure that the authorized labeling is made available to vaccination providers through appropriate

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means." (Emphasis added), and "Vaccination provider will administer the vaccine in accordance with the authorization." Therefore, it cannot be argued that the EUA Statute did not apply to Governor Inslee. Governor Inslee had a ministerial duty to "instruct" and "ensure" all individuals under his authority were informed of their lawful authority to accept or refuse (21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III)) and accepting the individual's chosen option was a purely ministerial option requiring no discretionary thought.

Defendant's assertion that "Plaintiffs could have complied with the Proclamation by obtaining the fully authorized Pfizer vaccine, which is not an 'emergency use' product subject to the statute 21 U.S.C. § 360bbb-3(e)(1)(A)," fails for reasons previously discussed. His contention that the Statute does not apply fails for reasons previously discussed. However, Defendant admits that the HHS Secretary must "establish conditions" for the EUA product, but provides no source of authority allowing Defendant to amend by Proclamation that which the HHS Secretary established.

Moreover, citing the DOJ's slip opinion is dangerous. First, the slip opinion's author advised that a person can misbrand an investigational new drug as though it were FDA-licensed. Second, the opinion suggests others can usurp from Congress

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its authority to prohibit involuntary participation in products authorized under the statute—an authority not even granted to the HHS Secretary (21 U.S.C. §360bbb-3(l)).

Plaintiffs allege that Governor Inslee was required to "instruct" and "ensure" Plaintiffs rights were recited to them and protected by him according to his ministerial duties under the CDC Program, Fourteenth Amendment, and the EUA Statute.

c. Enforceable rights

45 C.F.R. Part 46 was promulgated at the express directive of the U.S. Congress under the National Research Act, requiring every appropriated penny of the federal budget to comply with its requirements and persons using that funding under its authority. The Belmont Report is a requirement of 45 C.F.R. Part 46 when federal funds involve humans with investigational new drugs. The State's FWA applies because the State agreed to comply with 45 C.F.R. Part 46 and the Belmont Report as a condition to receive its FWA authorizing it to participate in federal funding.

i. 45 C.F.R. § 46

Governor Inslee's duties under the Provider Agreement meet the definition of

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conducting "research" on behalf of the federal government as described in Plaintiffs' Complaint requiring the use of the State's FWA. Governor Inslee was involved in a federally funded research activity, irrespective of whether he expended federal funds. However, the federal government provided the state with grants for those activities and private parties funding for each administered shot.

ii. The Belmont Report

The State agreed to comply with the Belmont Report and 45 C.F.R. § 46.116, forming the only statutory basis of what the federal government considers "Legally Effective Informed Consent." The CDC Program was not a clinical trial but nevertheless met the definition of research for purposes of 45 C.F.R. § 46.

iii. The FWA

The Proclamation requires all persons to participate in the federal government's COVID-19 investigational new drugs administered under research conditions requiring Governor Inslee to comply with the State's FWA.

d. Due Process

i. Substantive Due Process Claims

"Substantive due process protects individuals from arbitrary deprivation of their liberty by government." *Sylvia Landfield Tr. v. City of Los Angeles*, 729 F.3d

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1189, 1195 (9th Cir. 2013). The Proclamation is an arbitrary deprivation of Plaintiffs' liberty interests (bodily autonomy and integrity, right to be free from forced medical procedures, right to use their personal property (i.e., state-issued healthcare licenses)) and Gov. Inslee had no rational basis to issue it. First, the Proclamation relied exclusively on federally funded COVID-19 drugs for compliance. Second, as applied, the Proclamation interfered with the federal government's goals under the EUA statute and the PREP Act and was preempted by the Supremacy Clause under the EUA statute and the express preemption language under the PREP Act. Third, Congress prohibited the HHS Secretary from requiring anyone to participate in any activity authorized under the statute. Fourth, Gov. Inslee admits issuing his Proclamation on August 9, 2021, which is four days after CDC Director Rochelle Walensky admitted that the COVID-19 EUA/PREP Act investigational drugs did not prevent infection or transmission. Thus, if Gov. Inslee's goal was to "reduce the spread of COVID-19," there is no rational basis to use a drug that does not prevent infection or transmission. Therefore, the Proclamation was without legal authority, was arbitrary, and was not rationally based. The Proclamation was an "egregious" act because Governor Inslee:

(1) violated his voluntary ministerial duties under the CDC Agreement,

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(2) violated his ministerial duties under each EUA,
(3) violated his ministerial duties under the State's FWA,
(4) mandated that each healthcare facility commit fraud against the federal government by engaging in "other administration" of the federal property,
(5) concealed the material fact that Plaintiffs were authorized by Congress to refuse the federal property without consequence and required "vaccination providers" to terminate Plaintiffs' careers, finances, emotional well-being, and lifelong dreams.

The right to refuse an investigational new drug is aptly demonstrated in Congress enacting the 1974 National Research Act, promulgating 45 C.F.R. Part 46, establishing institutional review boards, ratifying Article VII ICCPR Treaty, establishing its FWA program, establishing the right to refuse under 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III), and denying the HHS Secretary authority to require compulsory participation in an investigational product under emergency expanded access protocols. The right to refuse an investigational new drug is a fundamental right "deeply rooted in this Nation's history and tradition."

ii. Procedural Due Process Claims

Plaintiffs have a constitutional right to have "an opportunity to present their objections" (*Milliken v. Meyer,* 311 U. S. 457 (1940)) and to have their "merits fairly judged" (*Logan v. Zimmerman Brush Co*., 455 U.S. 422 (1982)). Defendant did not inform Plaintiffs where, when, and how they could present their case. Federal law

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does not require plaintiffs to seek an exemption as a condition to refuse the federally funded COVID-19 EUA/PREP Act drug. Therefore, seeking an exemption was not part of any legitimate process available to Plaintiffs. Moreover, at all times pertinent, Defendant refused to acknowledge Plaintiff's federal right to refuse, and no process allowed their "merits" to be "fairly judged."

e. Equal Protection

"The Equal Protection Clause of the Fourteenth Amendment commands that no state shall deny to any person within its jurisdiction the equal protection of the laws, which is essentially a direction that all persons similarly situated should be treated alike." *City of Cleburne*, 473 U.S. at 439 (cleaned up). Defendant did not treat healthcare workers (Plaintiffs) choosing the option to refuse equally with healthcare workers choosing the option to accept. There was no rational reason why Defendant discriminated against Plaintiffs' rights when he voluntarily agreed to secure them on behalf of the federal government under the CDC Program and the EUA statute. The Proclamation differentiated between those who chose the option to accept and those who chose to refuse in violation of the Fourteenth Amendment's Equal Protection Doctrine.

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f. Spending Clause

While the Spending Clause itself does not create a cause of action, the conferred rights in any spending legislation do. The option to accept or refuse under the EUA statute applies to any person (Governor Inslee and Shriners) engaged in the CDC Program because the federal government fully funds that program. As such, it is spending legislation, so deprivation of the conferred rights under the EUA statute provides the basis for a § 1983 action.

The federal government purchased all COVID-19 drugs using DoD's funding authority. The ministerial duty to obtain informed consent means an individual has the explicit right to give it. Governor Inslee agreed to the terms of the Provider Agreement and the EUA statute relying on DoD-funded products under research conditions requiring him to "ensure" Plaintiffs gave their informed consent.

Defendant's statement that the CDC Program and its reliance on the EUA statute and 10 U.S.C. § 980 does not meet the Supreme Court's threshold for a § 1983 cause of action fails in light of *Talevski*.

g. Unconstitutional Conditions Doctrine

Defendant's claim that the Proclamation did not establish an Unconstitutional Condition fails because he required Plaintiffs to (1) surrender their legal right to

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refuse under the EUA statute, (2) surrender their due process rights to seek judicial relief for PREP Act injuries, and/or (3) surrender their right to refuse upon seeking a medical or religious exemption as a condition to continue employment under the state's licensing requirements. Such facts demonstrate an unconstitutional condition.

h. The PREP Act

The PREP Act "expressly" preempted Defendant's authority to issue his Proclamation as applied to the exclusive use of listed countermeasures under the Act because it is a "legal requirement" that "is different from, or is in conflict with, any requirement applicable under this section" and "relates to the...administration...of the covered countermeasure, or to any matter included in a requirement applicable to the covered countermeasure under this section or any other provision of this chapter, **or under the Federal Food, Drug, and Cosmetic Act** [21 U.S.C. 301 *et seq*.] (emphasis added). The option to refuse a product under the EUA statute is under 21 U.S.C. 301 *et seq*. (21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III)) containing the rights creating language required by the Supreme Court to create a cause of action under 42 U.S.C. § 1983.

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F. State Law Claims

Governor Inslee's Proclamation required private parties to violate well-established common law that no person can be discharged from employment exercising a legal right secured by a valid act of Congress.[5] Moreover, the Proclamation, as applied, was not a "reasoned exercise of the State's police power" for reasons previously discussed.

1. Breach of Contract

Regarding breach of contract, Defendant incorrectly contends that he is not a party to the Provider Agreement. The Provider Agreement states that the Vaccination Program is "part of collaboration under the relevant state, local, or territorial immunization's cooperative agreement with CDC," that "Organization must submit…data through…the immunization information system (IIS) of the state," that "Organization must comply with each relevant jurisdiction's immunization program guidance," and that "the jurisdiction's immunization program is required to create a unique COVID-19 ID for the organization." Defendant is the top executive of the State of Washington who volunteered the state

[5] *Becker v. Community Health Systems*, 184 Wn.2d 252, 359 P.3d 746 (2015); *Rose v. Anderson Hay & Grain Co*., 184 Wn.2d 268, 358 P.3d 1159 (2015), and *Rickman v. Premera Blue Cross*, 184 Wn.2d 300, 358 P.3d 1153 (2015)

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to participate in the Provider Agreement, required all parties to sign and abide by the Provider Agreement, and was responsible for fulfilling the State's requirements under the Provider Agreement. Governor Inslee's Proclamation breached the Provider Agreement because it violated Plaintiffs' third-party beneficiary rights, such as the right to be informed of the option to accept or refuse (which Defendant failed to do), the right to consider that participation without fearing or being subjected to "sanctions," "coercion," or "undue influence, and to consider participation by obtaining medical counseling from participating providers without incurring a fee.

2. Outrage Claim

Defendant (1) volunteered the State to participate in the CDC Program, (2) required parties to sign the Provider Agreement, (3) concealed the material fact that the entire healthcare industry was free to refuse the federally funded COVID-19 property without consequence, (4) issued a Proclamation in defiance of his voluntary assurances and ministerial duties, (5) stole from Plaintiffs a lifetime of achievement in their chosen profession when exercising their option to refuse. The "average" laborer would consider such an act outrageous, extreme, and morally debased character.

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G. Implied Private Right of Action

The same judicial reasoning led courts to imply a private right of action under Title VI and IX is identical in nature to the right under the EUA statute.

H. Qualified Immunity

Defendant is not entitled to qualified immunity. "A person 'subjects 'another to the deprivation of a constitutional right, within the meaning of section 1983, if he does an affirmative act, participates in another's affirmative acts, or omits to perform an act which he is legally required to do that causes the deprivation of which complaint is made." *Johnson v. Duffy*, 588 F.2d 740, 743 (9th Cir. 1978) citing *Sims v. Adams*, 537 F.2d 829 (5th Cir. 1976). Defendants enacted the vaccine policy, executed the vaccine policy, and enforced the vaccine policy by terminating the employment of any person exercising their option to refuse.

"As a matter of public policy, qualified immunity provides ample protection to all but the plainly incompetent or those who knowingly violate the law (*Malley v. Briggs*, 475 U.S. 335 (1986)). Defendant was an "emergency response stakeholder" required to perform for the State when the State volunteered to distribute the federal COVID-19 drug property under the CDC Agreement and applicable laws. When discussing any matters pertaining to the Pfizer-BioNTech COVID-19 Vaccine EUA

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drug, Defendant was to "instruct" "vaccination providers" on how to "administer the vaccine under the EUA." To administer means not only physical injection but also ensuring individuals are informed of their right to accept or refuse the product without penalty. Moreover, Defendant required all medical facilities to sign the CDC Agreement, whereupon those parties agreed to comply with "any EUA." Therefore, it cannot be reasonably argued that Defendant did not knowingly violate federal law when issuing his Proclamation relying on the EUA/PREP Act products offered only under the CDC Agreement. Moreover, Defendant has ministerial duties under the State's FWA, requiring him to ensure individuals give their informed consent anytime the State involves individuals with investigational new drugs funded by the federal government.

Defendant cannot claim qualified immunity on the basis that he was unaware of the legal requirements of medical facilities operating under the CDC Agreement.

The CDC Agreement required adherence to the EUA statute and "any EUA" demonstrating that Defendant lacked any discretionary authority to issue a Proclamation requiring Plaintiffs to inject the federally funded drugs into their bodies as a condition of continued employment. Instead, Defendant was under

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ministerial [6] duties to "instruct" "vaccination providers" on the "proper administration" of the federal COVID-19 EUA drugs "consistent with the terms of this letter and CDC's COVID-19 Vaccination Program."[7] The Provider Agreement is unambiguous in its intent when it states, "Organization must comply with all applicable requirements as set forth by the U.S. Food and Drug Administration, including but not limited to requirements in any EUA that covers COVID-19 Vaccine," duties to "instruct" Shriners of Plaintiffs' rights and "inform" Plaintiffs of their own rights. Still, to this day, Defendant refuses to educate the public on its right to reject an EUA/PREP Act product without penalty for all existing EUAs, which the State is lawfully bound to ensure. Moreover, Defendant was under a ministerial duty to accept a person's chosen option and could not deny employment to an individual within the State's healthcare industry choosing the option to accept or

[6] The Sixth Circuit held that the City of Dayton's "board members" were not entitled to qualified immunity when terminating a city employee, holding that board members could not "cloak themselves in [qualified] immunity simply by delegating their termination procedure decisions to their legal department….A reasonable competent public official is presumed to know the law governing his or her conduct" (*Silberstein v. City of Dayton*, 440 F.3d 306 (6th Cir. 2006)). The Tenth Circuit held that qualified immunity is not a defense for an official that "failed to actually comply with the statute upon which they purportedly relied" (*Roska v. Sneddon*, 437 F.3d 964 (10th Cir. 2006)). The Eleventh Circuit granted qualified immunity to a teacher who reprimanded a child for raising a fist in the air during the pledge of allegiance because the teacher was acting under *discretionary authority* but denied her that same immunity when encouraging children to pray because her actions were outside the scope of her authority (*Holloman v. Harland*, 370 F.3d 1252 (11th Cir. 2004)).

[7] See Doc 1-2, Letter P; Doc 1-2, FN 13

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refuse.[8] There is no discretionary function when it comes to accepting a person's chosen option under the EUA statute, so Defendant's claim for qualified immunity is unfounded.

The Defendant issued his Proclamation under *ultra vires* authority. Then, he required other parties to violate the Plaintiffs' constitutional and statutory rights, even though those parties signed a contract promising not to engage in the unlawful conduct Defendant mandated (such conduct constitutes fraud as outlined in the CDC Agreement). Defendant's Proclamation cannot reconcile with the EUA statute, the CDC Program, the Provider Agreement, the PREP Act, the Supremacy Clause, any EUA, or the State's FWA, all of which deny Defendant the right to a qualified immunity defense. Moreover, the Supreme Court held:

> "[G]eneral statements of the law are not inherently incapable of giving fair and clear warning, and in other instances, a general constitutional rule already identified in the decisional law may apply with obvious clarity to the specific conduct in question, even though 'the very action in question has [not] previously been held unlawful'" and "[T]he object of the 'clearly established' immunity standard is not different from that of 'fair warning' as it relates to law 'made specific' for the purpose of validly applying §242. The fact that one has a civil and the other a criminal law role is of no significance; both serve the same

[8] The Ninth Circuit held that officials were only engaged in ministerial acts unprotected from qualified immunity when refusing to allow plaintiffs to apply for a license (*Groten v. California*, 251 F.3d 844 (9th Cir. 2001)).

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> objective, and in effect the qualified immunity test is simply the adaptation of the fair warning standard to give officials (and, ultimately, governments) the same protection from civil liability and its consequences that individuals have traditionally possessed in the face of vague criminal statutes. To require something clearer than 'clearly established' would, then, call for something beyond 'fair warning.'[9]

Defendant does not cite a statute upon which he asserts a qualified immunity defense. Defendant only claims qualified immunity for issuing a vaccine mandate, which Plaintiffs do not contest. Defendant knew or should have known that well-established law dictates that a person cannot promote an unlicensed drug as though it were licensed, [10] such as Defendants' Proclamation when treating the investigational drugs as if they were licensed by the FDA for general commercial marketing with a legal indication to treat a known virus. However, none of the available federal COVID-19 drugs under the Defendants' vaccine policy bore such a label.

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[9] *United States v. Lanier*, 520 U.S. 259 (1997), citing *Anderson v. Creighton*, 483 U.S. 635 (1987)

[10] In 2009, the FDA charged Pfizer with a felony for promoting four drugs outside of their legal indication and fined them over $1.19 billion. The Department of Justice noted, "Under the provisions of the Food, Drug and Cosmetic Act, a company must specify the intended uses of a product in its new drug application to FDA. Once approved, the drug may not be marketed or promoted for so-called "off-label" uses – i.e., any use not specified in an application and approved by FDA."- Justice Department Announces Largest Health Care Fraud Settlement in Its History. Justice.gov. Published September 2, 2009

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IV. ORAL ARGUMENT REQUESTED

Plaintiffs believe oral argument would be helpful and hereby request same.

V. CONCLUSION

For the reasons set forth above, Plaintiffs urge the court to deny Gov. Jay Inslee's Motion to Dismiss, in its entirety.

RESPECTFULLY SUBMITTED,

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COVID-19 Vaccine Related Codes

COVID-19 Vaccine Codes

Preview Posting of COVID-19 Vaccine Codes and Crosswalk for Currently Authorized Vaccines and Anticipation of Potential Vaccine Availability under Emergency Use Authorization (EUA)

Note: Codes will become effective only upon EUA issuance or BLA licensure of COVID-19 vaccine(s) by the Food and Drug Administration (FDA)

The codes and crosswalk for candidate COVID-19 vaccines will be posted for preview in phases as the late-stage clinical trials for candidate vaccines progress. Additional vaccines or codes will be added to this list as they enter late-stage clinical trials or prepare applications for FDA authorization.

The following downloadable table provides a summary of the currently authorized vaccine codes and a preview of the vaccine codes that will be activated if the FDA authorizes use and ACIP votes to recommend the candidate vaccines.

To support this effort, the CDC is working closely with data partners responsible for the creation and distribution of vaccine codes and drug compendia publishers to coordinate the release of codes in advance of potential EUAs to enable systems and users that require these codes to prepare in advance.

The codes for these vaccines are also included in the vaccine code set files unless otherwise noted in the table. Additional code details and fields values are included in the vaccine code sets.

American Medical Association (AMA) COVID-19 CPT® vaccine product and administration codes are now available on the AMA web site. The CPT vaccine product codes are included in the Preview COVID-19 table and the CDC vaccine code sets. You can access further information regarding the COVID-19 CPT codes, as well as the associated coding guidance, using the following link:

https://www.ama-assn.org/practice-management/cpt/covid-19-cpt-vaccine-and-immunization-codes

Download the Preview Table *for US vaccine administration only*: Excel Version

COVID-19 vaccine codes and crosswalks are provided in anticipation of potential vaccine availability under an approved Biologics License Application (BLA), Emergency Use Authorization (EUA), or as a potential vaccine submission for EUA (Pre-EUA). Codes will become effective for US vaccine administrations only upon EUA issuance and/or BLA approval of COVID-19 vaccine(s) by the FDA. All CVX codes are associated to the new Vaccine Group "COVID-19." CPT codes shown are product codes. CPT administrative codes for doses are available on the AMA website. CPT product codes are added as the AMA approves and makes them available.

Manufacturer	FDA Authorization (BLA, EUA, Pre-EUA)	Sale Proprietary Name	Product Description	Unit of Sale NDC10 (UOS)	UoS Package	Unit of Use NDC10 (UOU)	UoU Presentation	CVX Code	CVX Long Description	CVX Short Description	MV Cod
Janssen Products, LP	EUA-authorized (18+)	Janssen COVID-19 Vaccine (BLUE CAP)	5x10^10 viral particles/0.5 mL for adult 18+	59676-580-15	CARTON, 10 MULTI-DOSE VIALS	59676-580-05	VIAL, MULTI-DOSE	212	SARS-COV-2 (COVID-19) vaccine, vector non-replicating, recombinant spike protein-Ad26, preservative free, 0.5 mL	COVID-19 vaccine, vector-nr, rS-Ad26, PF, 0.5 mL	JSN
Moderna US, Inc.	EUA-authorized (12+)	Moderna COVID-19 Vaccine (RED CAP)	100 mcg/0.5 mL for adult primary series and IC dose 12+ (existing product)	80777-273-99	CARTON, 10 MULTI-DOSE VIAL 5 mL EACH	80777-273-10	VIAL, 5 mL, MULTI-DOSE VIAL	207	SARS-COV-2 (COVID-19) vaccine, mRNA, spike protein, LNP, preservative free, 100 mcg/0.5mL dose or 50 mcg/0.25mL dose	COVID-19, mRNA, LNP-S, PF, 100 mcg/0.5mL dose or 50 mcg/0.25mL dose	MC
				80777-	CARTON,	80777-	VIAL, 7 mL,				

Exhibit A

Manufacturer	FDA Authorization (BLA, EUA, Pre-EUA)	Sale Proprietary Name	Product Description	Unit of Sale NDC10 (UOS)	UoS Package	Unit of Use NDC10 (UOU)	UoU Presentation	CVX Code	CVX Long Description	CVX Short Description	MV Coc
Moderna US, Inc.	EUA-authorized	Moderna COVID-19 Vaccine Booster (BLUE CAP)	New concentration, 50 mcg/0.50 mL for booster dose adults 18+ yrs, also authorized for primary series and IC doses for ages 6 years to <12 years. On 8/31/2022, EUA rescinded adult booster dose from this vial. EUA continues for 6 years to <12 year primary and IC doses.	80777-275-99	CARTON, 10 MULTI-DOSE VIAL 2.5 mL EACH	80777-275-05	VIAL, 2.5 mL, MULTI-DOSE VIAL	221	SARS-COV-2 (COVID-19) vaccine, mRNA, spike protein, LNP, preservative free, 50 mcg/0.5 mL dose	COVID-19, mRNA, LNP-S, PF, 50 mcg/0.5 mL dose	MC
Moderna US, Inc.	EUA-authorized for ages 6 months to < 6 years	Moderna COVID-19 Vaccine Pediatric Vaccine (BLUE CAP)	25 mcg/0.25 mL dose For pediatric primary series and IC doses for ages 6 mo to <6 years	80777-279-99	CARTON, 10 MULTI-DOSE VIAL 2.5 mL EACH	80777-279-05	VIAL, 2.5 mL, MULTI-DOSE VIAL	228	SARS-COV-2 (COVID-19) vaccine, mRNA, spike protein, LNP, preservative free, pediatric 25 mcg/0.25 mL dose	COVID-19, mRNA, LNP-S, PF, pediatric 25 mcg/0.25 mL dose	MC
Moderna US, Inc.	EUA Authorized for ages 18 yrs and older	Moderna COVID-19 Vaccine Bivalent Booster for ages 6 + years (18+, 12-17, 6-11) as authorized)	0.5 mL adult dose Original and Omicron BA.4 & BA.5 COVID-19 strains	80777-282-99	CARTON, 10 MULTI-DOSE VIALS	80777-282-05	VIAL, 2.5 mL, MULTI-DOSE VIAL	229	SARS-COV-2 (COVID-19) vaccine, mRNA, spike protein, LNP, bivalent booster, preservative free, 50 mcg/0.5 mL or 25mcg/0.25mL dose	COVID-19, mRNA, LNP-S, bivalent booster, PF, 50 mcg/0.5 mL or 25mcg/0.25mL dose	MC
Moderna US, Inc.	Pre-EUA Authorization for ages < 18 yrs		0.25 mL dose for 6-11 yrs of Original and Omicron BA.4 & BA.5 COVID-19 strains								
Pfizer-BioNTech	EUA-authorized for ages 12 yrs +	Pfizer-BioNTech COVID-19 Vaccine (PURPLE CAP) (Original product formulation)	30 mcg/0.3 mL for primary series, IC 3rd dose and boosters. EUA for Booster dose of this formulation has ended. After 8/31/2022, only bivalent boosters should be administered to ages 12+ years.	59267-1000-2	CARTON, 195 MULTI-DOSE VIALS	59267-1000-1	MULTI-DOSE VIAL	208	SARS-COV-2 (COVID-19) vaccine, mRNA, spike protein, LNP, preservative free, 30 mcg/0.3 mL dose	COVID-19, mRNA, LNP-S, PF, 30 mcg/0.3 mL dose	PFR
				59267-1000-3	CARTON, 25 MULTI-DOSE VIALS						

Manufacturer	FDA Authorization (BLA, EUA, Pre-EUA)	Sale Proprietary Name	Product Description	Unit of Sale NDC10 (UOS)	UoS Package	Unit of Use NDC10 (UOU)	UoU Presentation	CVX Code	CVX Long Description	CVX Short Description	MV Coc
				0069-2025-25	CARTON, 25 MULTI-DOSE VIALS						
Pfizer-BioNTech	EUA-authorized for ages 5 yrs to < 12 yrs	Pfizer-BioNTech COVID-19 Vaccine (ORANGE CAP) (Tris-sucrose formulation)	10 mcg/0.2 mL for primary series, IC 3rd dose, booster	59267-1055-4	CARTON, 10 MULTI-DOSE VIALS	59267-1055-1	VIAL, 2 mL, MULTI-DOSE VIAL	218	SARS-COV-2 (COVID-19) vaccine, mRNA, spike protein, LNP, preservative free, 10 mcg/0.2 mL dose, tris-sucrose formulation	COVID-19, mRNA, LNP-S, PF, 10 mcg/0.2 mL dose, tris-sucrose	PFR
Pfizer-BioNTech	EUA authorized for ages 6 mo to <5 yrs	Pfizer-BioNTech COVID-19 Vaccine (MAROON CAP) (Tris-sucrose formulation)	3 mcg/0.2 mL for primary series	59267-0078-4	CARTON, 10 MULTI-DOSE VIALS	59267-0078-1	VIAL, 2 mL, MULTI-DOSE VIAL	219	SARS-COV-2 (COVID-19) vaccine, mRNA, spike protein, LNP, preservative free, 3 mcg/0.2 mL dose, tris-sucrose formulation	COVID-19, mRNA, LNP-S, PF, 3 mcg/0.2 mL dose, tris-sucrose	PFR
Pfizer-BioNTech	EUA Authorized	Pfizer-BioNTech COVID-19 Vaccine Bivalent Booster 12 year and older (GRAY CAP) (Tris-sucrose formulation)	30 mcg/0.3 mL dose Original and Omicron BA.4 & BA.5 COVID-19 strains	59267-0304-2	CARTON, 10 MULTI-DOSE VIALS	59267-0304-1	VIAL, 2 mL, MULTI-DOSE VIAL Each vial contains 6 doses of 0.3 mL	300	SARS-COV-2 (COVID-19) vaccine, mRNA, spike protein, LNP, bivalent booster, preservative free, 30 mcg/0.3mL dose, tris-sucrose formulation	COVID-19, mRNA, LNP-S, bivalent booster, PF, 30 mcg/0.3 mL dose	PFR
				59267-1404-2	CARTON, 10 SINGLE-DOSE VIALS	59267-1404-1	VIAL, 1 dose of 0.3 mL				
Pfizer-BioNTech	Pre-EUA Authorization	Pfizer-BioNTech COVID-19 Vaccine Bivalent Booster 5 years through 11 years (ORANGE CAP) (Tris-sucrose formulation)	10 mcg/0.2 mL dose Original and Omicron BA.4 & BA.5 COVID-19 strains	59267-0565-2	CARTON, 10 MULTI-DOSE VIALS	59267-0565-1	VIAL, MULTI-DOSE Each vial contains 10 doses of 0.2 mL	301	SARS-COV-2 (COVID-19) vaccine, mRNA, spike protein, LNP, bivalent booster, preservative free, 10 mcg/0.2 mL dose, tris-sucrose formulation	COVID-19, mRNA, LNP-S, bivalent booster, PF, 10 mcg/0.2 mL dose	PFR

Manufacturer	FDA Authorization (BLA, EUA, Pre-EUA)	Sale Proprietary Name	Product Description	Unit of Sale NDC10 (UOS)	UoS Package	Unit of Use NDC10 (UOU)	UoU Presentation	CVX Code	CVX Long Description	CVX Short Description	MV Coc
Sanofi Pasteur	Pre-EUA Authorization	Sanofi Pasteur COVID-19 Vaccine, booster dose, adult	5mcg/0.5mL dose, including added AS03 adjuvant, booster dose only	49281-618-20	10 MULTI-DOSE VIALS, ANTIGEN ONLY	49281-618-78	VIAL, MULTI-DOSE, ANTIGEN	225	SARS-COV-2 (COVID-19) vaccine, D614, prefusion spike recombinant protein subunit (CoV2 preS dTM), AS03 adjuvant added, preservative free, 5mcg/0.5mL dose	COVID-19, D614, recomb, preS dTM, AS03 adjuvant add, PF, 5mcg/0.5mL	PM

Unspecified US COVID-19 Vaccine CVX Code

CVX Short Description	CVX Code	CVX Long Description	Note	Vaccine Status
SARS-COV-2 (COVID-19) vaccine, UNSPECIFIED	213	SARS-COV-2 (COVID-19) vaccine, UNSPECIFIED	Unspecified code for COVID-19 not to be used to record patient US administration. May be used to record historic US administration if product is not known. CVX code 500 should be used to record Non-US vaccine where product is not known.	Inactive

The following vaccine NDCs and associated tradenames have been either submitted for FDA authorization (Pre-Authorization) or have been authorized or approved by the FDA under EUA or BLA License and may be included in FDA NDC files and Structured Product Labels (SPL). These vaccines are listed separately because they represent NDCs that will not be manufactured or made available in the near term even if authorized.

Manufacturer	FDA Authorization (BLA, EUA, Pre-EUA)	Sale Proprietary Name	Product Description	Unit of Sale NDC10 (UOS)	UoS Package	Unit of Use NDC10 (UOU)	UoU Presentation	CVX Code	
Pfizer-BioNTech	BLA-licensed for ages 16+	COMIRNATY	30 mcg/0.3 mL for adult 16+ (original formula)	0069-1000-02	CARTON, 195 MULTI-DOSE VIALS	0069-1000-01	VIAL, 2 mL, MULTI-DOSE VIAL	**NDCs not included in CDC vaccine code set**	Pfizer Statement: "Pfizer received inital FDA BLA license on 8/23/2021 for its COVID-19 vaccine for use in individuals 16 and older (COMIRNATY). At that time, the FDA published a BLA package insert that included the approved new COVID-19 vaccine tradename COMIRNATY and listed 2 new NDCs (0069-1000-03, 0069-1000-02) and images of labels with the new tradename. These NDCs will not be manufactured. Only NDCs for the subsequently BLA approved tris-sucrose formulation will be produced."
				00069-1000-03	CARTON, 25 MULTI-DOSE VIALS				
Moderna US, Inc.	BLA-licensed for ages 18+	SPIKEVAX	0.5 mL dose (same as original EUA formula)	80777-100-98	CARTON, 10 MULTI-DOSE VIALS, EACH VIAL CONTAINING 7.5 mL	80777-100-15	VIAL, 7.5 mL, MULTI-DOSE VIAL	**NDCs not included in CDC vaccine code set**	"The following SPIKEVAX products are not anticipated to be manufactured and orderable. These NDC codes are not included in CDC Vaccine Code Set files at this time. Moderna has provided the following statement regarding the SPIKEVAX branded NDCs and labels: "Moderna received FDA BLA license on January 31, 2022, for its COVID-19 vaccine SPIKEVAX (COVID-19 Vaccine, mRNA) for use in individuals 18 and older. At that time, the FDA published a BLA package insert that included the new approved trade name SPIKEVAX and listed 2 new NDCs for Unit of Sale cartons (80777-100-99 and 80777-100-98). The NDCs related to the Carton of 10 7.5mL vials (80777-100-98/80777-100-15) will not be manufactured. Only the SPIKEVAX NDCs 80777-100-99 and 80777-100-11 will be manufactured at this time."
Moderna US, Inc.	EUA-authorized for ages 6 years to < 12 years	Moderna COVID-19 Vaccine Pediatric Vaccine (BLUE CAP)	50 mcg/0.50 mL pediatric primary series for ages 6 yrs to <12 years	80777-277-99	CARTON, 10 MULTI-DOSE VIAL 2.5 mL EACH	80777-277-05	VIAL, 2.5 mL, MULTI-DOSE VIAL	**NDCs and CVX Code 227 are currently included in CDC vaccine code set but have been inactivated. Moderna vaccines administered for**	Moderna Statement: "These codes have been provided in anticipation of FDA authorization and need. At this time, even though FDA authorization may be granted under EUA, supply is not foreseen in the short term. Moderna is actively assessing demand signals to activate supply plans and working closely with USG partners to collect official demand requirements."

CVX Code	CVX Long Description	CVX Short Description	CVX Note	CVX Status	MVX Code	MVX Manufacturer	Product Tradename(s)
208	SARS-COV-2 (COVID-19) vaccine, mRNA, spike protein, LNP, preservative free, 30 mcg/0.3mL dose	COVID-19, mRNA, LNP-S, PF, 30 mcg/0.3 mL dose	EUA 12/11/2020, 2-dose vaccine. Used to record Pfizer vaccines administered in the US and in non-US locations (includes tradename Comirnaty)	Active	PFR	Pfizer, Inc	Pfizer-BioNTech COVID-19 Vaccine (US-EUA), COMIRNATY (US-BLA), COMIRNATY (Non-US)
210	SARS-COV-2 (COVID-19) vaccine, vector non-replicating, recombinant spike protein-ChAdOx1, preservative free, 0.5 mL	COVID-19 vaccine, vector-nr, rS-ChAdOx1, PF, 0.5 mL	Potential EUA, 2-dose vaccine. AstraZeneca vaccine is authorized by the WHO and recognized towards immunity in the US. Non-US WHO authorized tradenames/identifiers include VAXZEVRIA, AZD1222, ChAdOx1 nCoV-19, COVISHIELD	Non-US	ASZ	AstraZeneca	AstraZeneca COVID-19 Vaccine (Non-US tradenames include VAXZEVRIA, COVISHIELD)
212	SARS-COV-2 (COVID-19) vaccine, vector non-replicating, recombinant spike protein-Ad26, preservative free, 0.5 mL	COVID-19 vaccine, vector-nr, rS-Ad26, PF, 0.5 mL	EUA 02/27/2021, 1-dose vaccine. Used to record Janssen/J&J vaccines administered in the US and in non-US locations	Active	JSN	Janssen	Janssen (J&J) COVID-19 Vaccine
510	SARS-COV-2 COVID-19 Inactivated Virus Non-US Vaccine Product (BIBP, Sinopharm)	COVID-19 IV Non-US Vaccine (BIBP, Sinopharm)	WHO authorized pandemic vaccine. Recognized towards immunity in US	Non-US	SPH	Sinopharm-Biotech	Sinopharm (BIBP) COVID-19 Vaccine
511	SARS-COV-2 COVID-19 Inactivated Virus Non-US Vaccine Product (CoronaVac, Sinovac)	COVID-19 IV Non-US Vaccine (CoronaVac, Sinovac)	WHO authorized pandemic vaccine. Recognized towards immunity in US	Non-US	SNV	Sinovac	Coronavac (Sinovac) COVID-19 Vaccine
512	SARS-COV-2 COVID-19 Virus Like Particle (VLP) Non-US Vaccine Product (Medicago, Covifenz)	SARS-COV-2 COVID-19 VLP Non-US Vaccine (Medicago, Covifenz)	Pandemic Non-US Vaccine not Authorized by WHO – ACIP does recognize towards immunity in US	Non-US	MDO	Medicago, Inc	Covifenz
500	SARS-COV-2 COVID-19 Non-US Vaccine, Specific Product Unknown	COVID-19 Non-US Vaccine, Product Unknown	Pandemic Non-US Covid Administration – specific CVX or product unknown	Non-US			
501	SARS-COV-2 COVID-19 Inactivated Virus Non-US Vaccine Product (QAZCOVID-IN)	COVID-19 IV Non-US Vaccine (QAZCOVID-IN)	Pandemic Non-US Vaccine not Authorized by WHO – not counted toward immunity in US	Non-US			
502	SARS-COV-2 COVID-19 Inactivated Virus Non-US Vaccine Product (COVAXIN)	COVID-19 IV Non-US Vaccine (COVAXIN)	Pandemic Non-US Vaccine Authorized by WHO 11-3-2021, recognized toward immunity in US, https://extranet.who.int/pqweb/vaccines/who-recommendation-bharat-biotech-intemational-ltd-covid-19-vaccine-whole-virion	Non-US	BBI	Bharat Biotech International Limited	COVAXIN (Bharat) COVID-19 Vaccine
503	SARS-COV-2 COVID-19 Live Attenuated Virus Non-US Vaccine Product (COVIVAC)	COVID-19 LAV Non-US Vaccine (COVIVAC)	Pandemic Non-US Vaccine not Authorized by WHO – not counted toward immunity in US	Non-US			
504	SARS-COV-2 COVID-19 Viral Vector Non-replicating Non-US Vaccine Product (Sputnik Light)	COVID-19 VVnr Non-US Vaccine (Sputnik Light)	Pandemic Non-US Vaccine not Authorized by WHO – not counted toward immunity in US	Non-US			
505	SARS-COV-2 COVID-19 Viral Vector Non-replicating Non-US Vaccine Product (Sputnik V)	COVID-19 VVnr Non-US Vaccine (Sputnik V)	Pandemic Non-US Vaccine not Authorized by WHO – not counted toward immunity in US	Non-US			
506	SARS-COV-2 COVID-19 Viral Vector Non-replicating Non-US Vaccine Product (CanSino Biological Inc./Beijing Institute of Biotechnology	COVID-19 VVnr Non-US Vaccine (CanSino Biological Inc./Beijing Institute of Biotechnology)	Pandemic Non-US Vaccine not Authorized by WHO – not counted toward immunity in US	Non-US			
507	SARS-COV-2 COVID-19 Protein Subunit Non-US Vaccine Product (Anhui Zhifei Longcom Biopharmaceutical + Institute of Microbiology, Chinese Academy of Sciences)	COVID-19 PS Non-US Vaccine (Anhui Zhifei Longcom Biopharmaceutical + Institute of Microbiology, Chinese Academy of Sciences	Pandemic Non-US Vaccine not Authorized by WHO – not counted toward immunity in US	Non-US			
508	SARS-COV-2 COVID-19 Protein Subunit Non-US Vaccine Product (Jiangsu Province Centers for Disease Control and Prevention)	COVID-19 PS Non-US Vaccine (Jiangsu Province Centers for Disease Control and Prevention)	Pandemic Non-US Vaccine not Authorized by WHO – not counted toward immunity in US	Non-US			
509	SARS-COV-2 COVID-19 Protein Subunit Non-US Vaccine Product (EpiVacCorona)	COVID-19 PS Non-US Vaccine (EpiVacCorona)	Pandemic Non-US Vaccine not Authorized by WHO – not counted toward immunity in US	Non-US			
513	SARS-COV-2 COVID-19 Protein Subunit Non-US Vaccine Product (Anhui Zhifei Longcom, Zifivax)	SARS-COV-2 COVID-19 PS Non-US Vaccine (Anhui Zhifei Longcom, Zifivax)	Pandemic Non-US Vaccine not Authorized by WHO – not counted toward immunity in US	Non-US			
514	SARS-COV-2 COVID-19 DNA Non-US Vaccine Product (Zydus Cadila, ZyCoV-D)	SARS-COV-2 COVID-19 DNA Non-US Vaccine (Zydus Cadila, ZyCoV-D)	Pandemic Non-US Vaccine not Authorized by WHO – not counted toward immunity in US	Non-US			
515	SARS-COV-2 COVID-19 Protein Subunit Non-US Vaccine Product (Medigen, MVC-COV1901)	SARS-COV-2 COVID-19 PS Non-US Vaccine (Medigen, MVC-COV1901)	Pandemic Non-US Vaccine not Authorized by WHO – not counted toward immunity in US	Non-US			
516	SARS-COV-2 COVID-19 Inactivated Non-US Vaccine Product (Minhai	COV-2 COVID-19 Inactivated Non-US Vaccine Product	Pandemic Non-US Vaccine not Authorized by WHO – not counted toward immunity in US	Non-US			

https://www.fda.gov/emergency-preparedness-and-response/coronavirus-disease-2019-covid-19/comirnaty-and-pfizer-biontech-covid-19-vaccine#additional

The FDA issued its initial Emergency Use Authorization for the Janssen (Johnson & Johnson) COVID-19 vaccine on Saturday February 27, 2021. Subsequent EUA amendments and authorizations have followed. Information regarding the Janssen vaccines as well as both the EUA Provider Fact Sheet and the EUA Recipient and Caregiver Fact Sheets is now available on the following FDA web site link:

https://www.fda.gov/emergency-preparedness-and-response/coronavirus-disease-2019-covid-19/janssen-covid-19-vaccine#additional

The FDA issued its initial Emergency Use Authorization for the Novavax COVID-19 vaccine on Wednesday July 13, 2022. Information regarding the Novavax vaccine as well as both the EUA Provider Fact Sheet and the EUA Recipient and Caregiver Fact Sheets is now available on the following FDA web site link:

https://www.fda.gov/emergency-preparedness-and-response/coronavirus-disease-2019-covid-19/novavax-covid-19-vaccine-adjuvanted?utm_medium=email&utm_source=govdelivery#additional

The following downloadable VIS code files will include the new EUA Fact Sheet for Recipients records:

- Excel version of table

CVX Code	EUA Recipient/Caregiver Fact Sheet Description	Document Barcode String	Edition Date	Edition Status	HTML URL	PDF URL
207, 221, 229	COVID-19 Moderna Vaccine EUA Recipient-Caregiver Fact Sheet	25308869830003491121060l	8/31/2022	Current	https://www.cdc.gov/vaccines/covid-19/eua/modernatx.html	https://eua.modernatx.com/covid19vaccine eua/eua-fact-sheet-recipients.pdf
227, 221	COVID-19 Moderna EUA Recipient-Caregiver Fact Sheet – Pediatric 6yrs to <12yrs	25308869830005091122050l	6/17/2022	Current	https://www.cdc.gov/vaccines/covid-19/eua/moderna-children-6-years-under-12-years.html	https://www.cdc.gov/vaccines/covid-19/eua/moderna-children-6-years-under-1 years.pdf
228	COVID-19 Moderna EUA Recipient-Caregiver Fact Sheet – Pediatric 6mo to <6yrs	25308869830005161122050l	6/17/2022	Current	https://www.cdc.gov/vaccines/covid-19/eua/moderna-children-6-months-under-6-years.html	https://www.cdc.gov/vaccines/covid-19/eua/moderna-children-6-months-under years.pdf
208, 217, 300	COVID-19 Pfizer BioNTech EUA Recipient-Caregiver Fact Sheet – 12 years and older	25308869830003321121050l	8/31/2022	Current	https://www.cdc.gov/vaccines/covid-19/eua/pfizer.html	https://www.fda.gov/media/153716/downlc
218	COVID-19 Pfizer BioNTech EUA Recipient-Caregiver Fact Sheet-Pediatric 5yrs to <12 yrs	25308869830004241121100l	6/28/2022	Current	https://www.cdc.gov/vaccines/covid-19/eua/pfizer-children.html	https://www.fda.gov/media/153717/downlc
219	COVID-19 Pfizer BioNTech EUA Recipient-Caregiver Fact Sheet-Pediatric <5 yrs	25308869830004861122010l	6/28/2022	Current	https://www.cdc.gov/vaccines/covid-19/eua/pfizer-children-under-5-years.html	https://www.fda.gov/media/159313/downlc
212	COVID-19 Janssen Vaccine EUA Recipient-Caregiver Fact Sheet	25308869830003631121020l	5/5/2022	Current	https://www.cdc.gov/vaccines/covid-19/eua/janssen.html	https://www.janssenlabels.com/emergency-authorization/Janssen+COVID-19+Vaccine-Recipient-fact-sheet.pdf
211	COVID-19 Novavax EUA Recipient-Caregiver Fact Sheet	25308869830003701122010l	8/19/2022	Current	https://www.cdc.gov/vaccines/covid-19/eua/novavax.html	https://www.fda.gov/media/159898/downlc
225	COVID-19 Sanofi-GSK EUA Recipient-Caregiver Fact Sheet – Booster	25308869830005231122040l	6/1/2022	Current	https://www.cdc.gov/vaccines/covid-19/eua/sanofi.html	https://www.cdc.gov/vaccines/covid-19/eua/sanofi.pdf

***Edition Date represents the date of update printed on the actual fact sheet document published by the FDA. It may not be the same as the EUA authorization date or the date embedded in the Document Barcode String.**